

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Breastfeeding

Overview

Advice and support for women to breastfeed; advising women who are living with HIV about breastfeeding.

What you will learn

At the end of this topic, you will be better able to:

- discuss whether women in a community breastfeed their babies
- explain why breastfeeding is important and good for the baby
- explain when a mother should begin breastfeeding her newborn and how long a mother should breastfeed her infant
- explain how best to support mothers to breastfeed and how to help the family to support them
- advise what a mother living with HIV should do about breastfeeding

Tools

- Benefits of breastfeeding poster
- How to breastfeed poster
- Common problems with breastfeeding poster

Topic content

Breastfeeding is the very best food that a newborn can have up until the age of six months. For the first six months of life, a baby does not need any food other than breast milk to develop into a healthy child. Mothers in every country of the world – developed and developing – should learn the benefits of breastfeeding and strategies to breastfeed their babies.

Role of the volunteer

It is helpful to find out whether most women in the community breastfeed their babies, and if not, why they do not. This information helps eCBHFA volunteers to give advice in a sensitive way.

The eCBHFA volunteer's key role is to get mothers to give only breast milk to their baby for the first six months of its life and to consider continuing to give breast milk for two years or more, even after other foods are given. Breastfeeding is the best way to give babies all the good things their bodies need to grow and be healthy. Mothers and their families should be taught about why breastfeeding is good for the baby, the mother and the family.

Mothers can be taught how to breastfeed, and eCBHFA volunteers can help to teach about common problems such as calming many mothers' fear of not having enough milk for the baby, mothers' sore nipples or painful, swollen breasts.

Even women living with HIV who are managing their viral load should give only breast milk for the first six months of the baby's life, because this is the best way to improve the baby's chance of remaining healthy. If an HIV+ mother is unsure of her viral load or is not taking their ARVs regularly, they should discuss breastfeeding at the clinic with the nurse or doctor.

Acceptance of breastfeeding in the community

It is important to know whether most women in the community give breast milk to their babies. The answer to this question will help eCBHFA volunteers to be sensitive with mothers and their families and, if necessary, tell them that some of their beliefs about breastfeeding may need to be adjusted.

Questions to pose to the community:

- In our community, do people think mothers should breastfeed their babies? If yes, why? If no, why not?
- Do all mothers in our community breastfeed their babies? Why or why not?
- Who in our community breastfeeds their babies? For how long?
- Who does not breastfeed? What do they do instead?
- Who in your community gives advice about breastfeeding? About formula feeding?
- Are most babies given just breast milk? Or, for example, are they given water, milk formula or other things instead of, or as well as, breast milk? Is so, what are the other things?
- How soon after birth do most mothers start breastfeeding? For how long?
- Who decides that the mother will breastfeed (the mother, father, mother-in-law, etc.)?
- In our community, who can help mothers who have difficulties with breastfeeding?
- Are there beliefs in our community about certain foods or herbs that should be eaten or avoided when a mother is breastfeeding?

Breastfeeding support – Benefits of breastfeeding poster

The eCBHFA volunteers' role is to advise families about breastfeeding as the best way for mother and baby to be healthy. Volunteers help mothers to keep breastfeeding for as long as they are able. Female volunteers can help new mothers with any problems with breastfeeding, for example, fear of too little milk, sore nipples or painful breasts.

eCBHFA volunteers can also run breastfeeding support groups for breastfeeding mothers in the community. This will be a safe and private place where mothers can breastfeed while discussing and supporting each other and sharing key messages with groups of mothers.

Key messages to pass on to mothers and their families are:

- Breast milk is the best and only food or liquid a baby should be given for the first six months of life. It protects the baby from diseases. It is always clean and natural, is available on demand 24 hours a day and costs nothing, unlike milk formula which is very expensive.
- Breastfeeding makes the bond between mother and baby strong.
- Breastfeeding helps the mother's body recover faster after the birth.
- Giving only breast milk for the first six months after the birth means a woman is less likely to get pregnant during that time.
- Breastfeeding can continue for up to two years or more, even when the baby starts to eat other foods after six months.
- After six months, babies can continue with breast milk but the diet should be supplemented with other nutritious foods like mashed vegetables and fruits, soft protein or egg, etc.

The mother and her family should understand that breastfeeding should start within the first hour after birth. This first milk helps the baby to be healthy and grow well. For the first breastfeeding after birth, the milk is actually a watery yellowish fluid, called colostrum which is rich in vitamins and nutrients. Colostrum is an important food for the baby to fight infections and grow strong in their first day in the world. Colostrum also helps the newborn's body prepare to digest breast milk. Shortly after the first breastfeeding, the mother's breasts will start to become firmer and her milk becomes thicker and more bluish white in colour. The mother should know that the more the baby sucks, the more breast milk the mother's breasts produce so there should not be a worry about not making enough milk.

The mother and her family should know that the baby needs nothing else for nourishment except breastmilk for the first six months. The baby should NOT be given water, milk formula or anything else as they can harm the baby because these liquids might not be clean. They also lack the vitamins and nutrients that are in breast milk and which fight infection.

Reflection

List at least three ways breastfeeding is best for babies and mothers.

1. When should mothers begin breastfeeding after the baby is born?
2. How long should a mother give the baby only breast milk?
3. How long should a mother continue to breastfeed once the baby starts eating other foods?

Helping mothers to learn to breastfeed

Breastfeeding is natural, but many mothers need guidance to do it directly the first few times they try. It can take mothers some time to get used to it, especially if it is the mother's first baby or she has not breastfed before. It is normal for women to find breastfeeding difficult at first. Tell them to be patient and keep trying. Sometimes babies take some time or need help to take the breast in its mouth, which can be frustrating for both baby and mother. Volunteers can help to calm the mother and baby until the baby is able to suck from the breast.

When should mothers start breastfeeding?

Babies should be breastfed within the first hour after birth or as soon as possible after birth.

How to start breastfeeding – How to breastfeed poster

The mother should be comfortably semi-sitting up or lying on her side. The newborn can be placed on the mother's bare chest to encourage skin-to-skin contact between mother and baby or the baby can be placed lying on the bed facing the mother's breast if the mother is on her side. To help the baby find the breast, make sure the baby is facing the breast with his nose directly opposite the mother's nipple and his chin touching her breast. Check that the baby's neck is not twisted.

Now, with the baby on the mother's chest or lying next to the mother if she is on her side, hold the baby's body close to the mother's body, and make sure the baby's entire body is supported, not just the neck and shoulders. Wait until the baby's mouth is open wide and move the baby onto the mother's breast, aiming his lower lip well below the nipple. Look at the baby. He should start opening its mouth, sticking its tongue out, licking and rooting (making sucking movements with the mouth). The baby will try a few times to feed before finding the nipple and beginning to suck.

Note that it can be difficult for the baby to breathe if the breast is pressed too tightly against the baby's nose. Show the mother how to press two of her fingers against the breast in front of the baby's nose to hold the breast away from the baby's nose.

The baby should suck slowly and deeply with some pauses. Allow the baby to suck for as long as it wants (at least five to ten minutes on each breast or longer). The baby should suck from both breasts at each feed. The mother should allow the baby to breastfeed whenever the baby wants. This is called feeding "on demand". Most babies breastfeed eight to 12 times during a 24-hour period for 10 to 20 minutes on each breast.

You should remind mothers that the baby should not be given water, milk formula or anything else. Breastfeeding is enough. Also between feeds, the baby should not be given feeding bottles or pacifiers (dummies) to suck.

Reflection

1. Describe what a baby does when it is ready to breastfeed.
2. How often should a baby be breastfed?
3. How does the mother feel about breastfeeding her baby? How does breastfeeding benefit the baby? How does it benefit the mother?

What if a mother needs to be away from the baby for more than a few hours?

Breast milk can be pumped by squeezing the breast and collecting the milk in a clean sterile container or bottle for feeding to the baby when the mother is absent. When feeding the baby saved breast milk, the milk should be prepared to the proper temperature using a bottle with a nipple that has been boiled and cleaned.

Cleanliness of the bottle and nipple are very important.

Common problems with breastfeeding – Common problems with breastfeeding poster

Not enough milk

The most common worries that mothers have are “I do not have enough milk”, “my baby is not getting all that s/he needs” or “my milk is not good enough”. If the baby cries, mothers might worry that the baby is hungry. These fears can lead to babies being given water, milk formula or other food.

Tell the mother that:

- most mothers produce plenty of milk for their babies to grow healthy and strong
- the more often the baby sucks on the mother's breasts, the more milk will come
- breast milk is all that a baby needs for the first six months of life
- a baby who is wetting six to eight diapers a day is getting plenty of milk
- babies need and enjoy the warmth and closeness of their body next to their mother
- babies cry for many reasons other than hunger

Sore nipples

Tell the mother that:

- it is normal to feel some soreness in the first weeks of breastfeeding
- she can prevent soreness by making sure the baby's mouth is fully on the nipple
- she can put one finger gently in the baby's mouth when removing the baby from the breast. This stops the baby sucking before it is removed from the breast which can be painful
- she should let her nipples dry in the air after feeding and before covering her breast

If the nipples become sore, a mother can squeeze a bit of breast milk onto her finger and apply it to the nipple. This will help relieve some of the pain. Mothers can also change the position of the baby when he breastfeeds so that when the baby sucks pressure is put on different places. Some positions might include putting a pillow under the baby while they feed, holding the baby under the mother's arm while they feed, or lying on her side with the baby lying next to her while feeding. See the How to breastfeed poster for suggested positions.

Swollen breasts

Breastfeeding mothers can suffer from swollen breasts which are sore to the touch. The pain can be alleviated by:

- breastfeeding the baby often and regularly
- placing warm wet pads or cloths on the breasts after breastfeeding which can help the milk flow and reduce swelling

If the breasts become hot and very painful to touch, the mother may have an infection and should go to the clinic.

Flat or inverted nipples

If the woman's nipples are flat or inverted (not sticking out), a mother can carefully squeeze her nipples and pull them gently before putting the nipple in the baby's mouth.

Reflection

1. What are the most common problems women have with breastfeeding in your community?
2. What are some ways to support women when they have problems with breastfeeding?

Living with HIV and breastfeeding

The HIV virus can be contained in breast milk, and while an HIV-infected mother can pass on the virus to her baby during pregnancy, birth and through breastfeeding, the World Health Organization, UNICEF and others say that mothers living with HIV should breastfeed their babies with minimal risk of infecting their babies with the HIV virus. Breast milk is still the best way to have a healthy baby as long as the mother is regularly taking her medicine for HIV and her viral load is low. Only then, the baby's chances of not contracting HIV are high.

The mother living with HIV should not mix breast milk and milk formula because this practice can actually increase the risk of passing on the virus to the baby. Mothers should give only breast milk for the first six months.

Advise a mother living with HIV who is unsure whether or not to breastfeed to talk to a healthcare worker for advice and to keep taking their medicine as directed.

Reflection

1. If an HIV+ mother knows that her viral load is low and is regularly taking her medication, is breastfeeding recommended?
2. When should an HIV+ mother **not** breastfeed her infant?
3. Is there a risk that the baby will contract HIV from its HIV+ mother?
4. If an HIV+ mother is unsure of her viral load or if she has not been taking her ARVs, should she breastfeed?

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Care during pregnancy

Overview

Pregnant women and those who care for them learn about health care during pregnancy, why pregnancy checks are so important, how to stay healthy during pregnancy, signs to watch out for that may mean there is a problem during the pregnancy, and preparing to breastfeed immediately after birth.

What you will learn

At the end of this topic, you will be better able to:

- say why being checked is so important when a woman is pregnant and why she should make regular visits to a health worker during her pregnancy
- say what the health worker should do at each visit
- advise mothers and their families about getting ready for a safe birth
- give advice about what to eat to stay healthy during pregnancy
- prepare mothers to breastfeed once the baby is born
- advise who to put the mother in touch with in the community for care and support if problems arise

Tools

- Go to the clinic if...poster
- Healthy foods for pregnant women poster
- Birth plan checklist poster
- Disease prevention checklist

The role of the volunteer

eCBHFA volunteers have an important role in helping the woman and her family have a healthy pregnancy, birth and newborn baby by:

- advising a pregnant woman to cut down on her work and to make more time to rest
- advising her to have regular check-ups during her pregnancy
- teaching the woman and her family about the signs to watch out for in pregnancy that may mean there is a problem
- making sure women know where to go for help if they see any of the signs
- helping the family to make a plan for the birth and know what to do if there are problems
- showing the woman how to relax and manage her worries

Relax...

Pregnancy can be a stressful time. Give pregnant women time to talk about feelings, hopes for the future, how they feel about the coming child and any worries or fears about the birth. Share with her what is happening with the child moving inside her and talk about earlier pregnancies and what happened. If this is her first baby, she might need extra help to feel ready for the birth.

Volunteers may also teach some self-care tips to the mother in how to take time to care for her own needs, how to relax and ways to manage her stress. A pregnant woman's body is doing an amazing task of growing a baby inside her and to do that, her body must be allowed time to do its important work. She can do this by taking care of herself, feeding the body well and managing her stress levels.

First-time mothers may need some guidance on the stressfulness of the first few weeks of the baby's life. The mother's body is trying to heal while she must also care for a new baby and his needs 24 hours a day. By establishing a care and support lifeline for new mothers, volunteers can start to build new self-care habits that will help her to best prepare for the baby's arrival and manage the stress of those first few weeks.

Helping partners to be more supportive

Pregnancy can also be stressful for the partner of the pregnant woman. It can be a time of stress, uncertainty and sometimes can result in unhealthy behaviours including domestic violence. eCBHFA volunteers can support by listening to the concerns of the mother's partner or holding a support group for partners to discuss and manage their stress. The partner also needs to be aware of the growing pregnancy and how to best prepare for the baby's birth and what the mother and baby will need during and after pregnancy.

eCBHFA volunteers can provide support to mothers whose partners are not present or are not supportive. By conducting household visits and setting up a support calendar, pregnant mothers can receive the community support they need at such a stressful time.

Reflection

1. Why is it important for mothers to manage and reduce the stress they may feel?
2. How can mothers remind themselves of their responsibility to care for themselves?

Pregnancy health checks

Pregnancy is a time of great change for a woman's body. Because of the growing foetus or baby in her body, pregnant women can be at risk for anaemia, high blood pressure, pre-eclampsia and other health issues, all of which can harm both the baby and mother. Pregnant women should go for antenatal checks regularly with more visits as the birth draws closer. If a health issue is found, they should go more frequently based on the midwife's recommendation. Volunteers should tell the mother what she can expect when she visits a health worker for pregnancy checks. The health worker is checking that everything is going well with mother and baby.

At the first visit, the health worker asks about any health problems the woman has had in the past which may affect the pregnancy. At every visit, the health worker should check the record of visits to see which tests and treatments have been given. The health worker should then:

- check how many weeks the woman has been pregnant and whether this is her first child. If it is not her first baby, check whether there were any problems in past pregnancies.
- take the woman's blood pressure. If the blood pressure is too high, it may cause problems for the mother and her unborn child.
- ask about any bleeding and other signs that there may be a problem
- check if the woman has had her blood tested for anaemia which means that her blood needs more iron to feel less tired and stronger. Women with anaemia are more at risk of infections and other problems.
- check if the woman has had her blood group typed. Human blood may look the same but there are different kinds of rhesus or natural substances in the blood that cause them to fall into different groups. These are A, B, AB and O and each type can be "rhesus negative" or "rhesus positive". If the blood type is rhesus negative, it can cause problems in a second or later pregnancy. The health worker can help identify and address this problem.
- check the woman's weight and height to check whether she is gaining weight in a healthy way
- go over the birth plan with the mother
- give advice about good foods to eat and drinking lots of water, and check that she is taking iron/folic acid tablets to keep her blood strong
- advise the woman not to use tobacco or drink alcohol during her pregnancy because they can cause harm to the unborn child
- advise the woman not to take any medicines without asking the health worker first if it is safe for her unborn child
- tell about possible signs to watch out for during pregnancy that may mean there is a problem
- advise about breastfeeding and how to plan for when the baby is born
- discuss birth control options that the mother needs to consider once she gives birth to ensure proper spacing and time for her body to rest (see the Family planning topic).
- ask the woman if she has any questions or worries and listen to her answers
- tell her about check-ups after the baby is born to make sure all is well and that she is coping with being a new mother

Disease prevention checks – Disease prevention checklist

At the antenatal visits, the health worker will check that the pregnant woman is protected against diseases which can harm the unborn child. The health worker will:

- Check to see if the woman has received her injections against tetanus.
- Check to ensure that the woman has been tested for different diseases (Hepatitis B, HIV, syphilis and tuberculosis) which can harm the unborn child.

HIV

- For HIV+ mothers, check their viral load and ensure they are taking their medication to allow for breastfeeding. Mothers should continue their treatment throughout the pregnancy and beyond.
- Explain to HIV+ mothers that the health worker may recommend the newborn baby receive medicines to protect it from HIV infection while breastfeeding.

Malaria

- In areas where there is malaria, check that the woman is sleeping under a bed net every night (one that has been treated to kill insects) and that she is taking the correct medicine to stop malaria.
- In areas where there is malaria, when a woman is 13 weeks pregnant, she should start taking intermittent preventive treatment (ITP) medicine to prevent getting malaria which can harm her and her unborn child.

Dengue, chikungunya and Zika

- In areas where there is dengue, chikungunya or Zika, check that the woman is using insect repellent, using a condom when having sex, sleeping under a bed net when napping during the day and keeping her home and immediate environment free of mosquito breeding sites.
- If a mother shows symptoms of Zika, test for Zika infection and provide counselling and guidance on next steps.

If qualified to do so, the health worker should examine the woman's body to check whether the pregnancy is progressing normally. The health worker may also carefully feel the stomach to check the position of the unborn child and listen to the baby's heart using a special tool called a Pinard horn.

Timing of antenatal visits

Regular and ongoing pregnancy checks with a health worker should begin as early in the woman's pregnancy as possible, ideally no later than the fourth month and after three monthly menstrual periods have been missed. Eight visits overall would be best, but at least four would be good. The World Health Organization (WHO, 2016) says most mothers are advised to have visits with the health worker as follows:

Pregnancy visits	Timing
1	Up to 12 weeks
2	20 weeks
3	26 weeks
4	30 weeks
5	34 weeks
6	36 weeks
7	38 weeks
9	40 weeks

If a woman fails to go for her pregnancy checks, it is difficult to check the health of both the mother and the baby as they develop.

If the woman has not given birth by 41 weeks, she should follow the birth plan and get help to deliver the baby.

Reflection

1. Why are health pregnancy checks important?
2. What questions might the mothers be asked by the health worker?
3. How do health workers check the health of the mother and the baby?
4. Why is it important to talk about birth control and preventing or planning the next pregnancy while the woman is still pregnant?
5. Why does the health worker talk about breastfeeding before the baby is even born?
6. What precautions should an HIV+ pregnant woman take to ensure her baby does not contract HIV before and after the birth?
7. What precautions should a pregnant woman take to protect against malaria, dengue, chikungunya and Zika?
8. How often should a pregnant woman see a health worker during her pregnancy?

Signs that there may be a problem – Go to the clinic if...poster

Explain to the mother and her family that problems can happen at any time during the pregnancy. There are a number of signs that there may be problems, and it is important for the woman and her family to know these signs and where to go for help if they see any of them.

The signs are:

- any bleeding from the vagina, especially bright red blood
- bad headache
- blurred vision
- swelling in the hands, feet or face
- convulsions or fits (does not stop shaking)
- loss of consciousness/fainting
- a high fever
- heavy vaginal discharge
- vaginal discharge that is foul smelling
- sudden gush of clear, watery fluid
- severe pain in the belly
- difficulty breathing
- pain when passing urine
- severe vomiting or vomiting all the time
- very pale palms of hands or under the nails
- ulcers on the sex organs
- woman stops feeling the baby move inside her
- low dull backache
- intense pressure at the base of the belly

Tell the mother and her family where the nearest medical help can be found and how to get there. This should be one part of the birth plan that you can help each family to prepare.

It is not advised to try treatments that are not approved by your health worker as it may interfere with the health of the mother or baby. This may include traditional medicines or practices that your health worker may not feel is appropriate or healthy for mother or baby. Some traditional practices may be just fine – talk with your health worker to check before you take any action, eat or drink anything about which you are unsure.

Reflection

1. Why is it important to know the signs that there may be a problem during pregnancy?
2. Where should the woman go if there is a problem? What if the health worker or facility where the mother usually goes is unavailable?
3. It is advised that if a traditional healer wants you to try something but you are unsure of its safety for your baby, check with your health worker first. How can you manage a situation like this when it comes up? Role play different scenarios of how a pregnant woman might manage this situation with:
 - a. The healer
 - b. The loved one who advises the mother to try it.

Giving advice on what to eat and drink – **Healthy foods for pregnant women tool**

Give the mother and her family advice about which foods the mother should eat so that she stays healthy during her pregnancy and has a healthy baby. Eating the right foods also helps the mother when she is breastfeeding. Advise the mother to:

- eat more or “eat for two” during the pregnancy
- eat foods such as meat and eggs: these contain iron which helps her blood stay strong
- eat well and take extra iron because a woman loses blood after the birth
- take the iron and folic acid tablets given to her to help prevent anaemia
- eat greens and fruit for more energy (e.g. spinach, cabbage, pumpkins, mango, carrots, tomatoes, avocado, oranges, pineapple, banana, potato and sweet potato)
- eat foods that help to build strong bones and fight infections, such as beans, lentils, peas, nuts, milk, yogurt, cheese, fish, eggs, chicken, meat, maize, wheat and rice
- drink lots of water

Tell the mother to watch out for signs of anaemia, and to tell the health worker at the pregnancy clinic if she notices any of them:

- feeling of tiredness or exhaustion
- pale inner eyelids, tongue and under the nails
- dizziness, especially when getting up from a sitting or lying position
- fainting
- shortness of breath
- fast heartbeat
- weakness

Reflection

1. What kinds of food are best for a pregnant woman to eat and drink?
2. If a pregnant woman feels sick and refuses to eat, how can an eCBHFA volunteer help?
3. What are some of the warning signs that a mother may have anaemia or not enough iron in her blood?

Planning the birth

Birth plan – Birth plan checklist tool

A birth plan is a strategy about the different aspects involved in the birth. It helps the woman and the family think through all the necessary details in preparing for the birth, the time immediately after the birth and once the baby is home. It allows the family to prepare for the new baby and what decisions might need to be made as birth approaches.

The **Birth plan checklist** tool starts at the beginning of a pathway with the prompt “When is your baby due?” Knowing when the baby is due to come, an eCBHFA volunteer can help the mother plan her health checks, help her monitor her weight and the movements of the baby. As the birth date approaches, the volunteer will want to check in more regularly with the mother.

Each prompt in the checklist will be a way to check in on the details the new mother and her family need to consider as the baby’s arrival draws closer.

Volunteers can go through each step of the **Birth plan checklist** with a pregnant woman and keep a record of the responses to each prompt to track the details of each woman’s plan and concerns, and check that nothing in the plan has changed between visits.

These are the questions that a pregnant woman and her family need to consider:

- When is the baby due?
- Where will the baby be born? Home? Hospital? Elsewhere?
- If the baby is to be delivered at home, ask what is the plan in case of an emergency during delivery?
- Who will deliver the baby? Advise the parents to go to a hospital or clinic with a person skilled in helping to give birth, if possible.
- What things do you need before the birth, e.g. clean cloths, clothes?
- Who will help you during labour (husband, sister, mother, mother-in-law, friend)?
- Who will care for the rest of your family during the birth and immediately after?
- Where will you go if you see a sign that there might be a problem?
- How will you travel to the hospital or clinic to get help?
- How will you contact the health worker, hospital or clinic when you go into labour?
- How long will it take to travel to where you will deliver the baby?
- How much will it cost to get there, and how will the family pay to get there?
- How will the family save money for the birth in case there are problems?

Preparing for breastfeeding

See also the **Breastfeeding** topic of this module for more detail.

You should help the mother get ready for breastfeeding her new baby. Tell her and her family that breast milk is the only food that the baby should be given for the first six months. This helps the baby to grow and be healthy. Tell them that:

- Breast milk is the perfect food for the baby, always there, just the right temperature and free (milk made from a formula is expensive and not as good as breast milk).
- In places where water may not always be safe to drink, breastfeeding keeps the baby free from diarrhoea and prevents infections.
- Breastfeeding should begin within the first hour after the birth.
- The watery milk that comes in the first couple of days, called “colostrum”, protects the baby from disease.
- The baby should not be given water or anything else; the breast milk is all the baby needs.
- The more the baby sucks, the more milk the mother will produce.
- Mothers should be prepared to breastfeed the baby every two hours or at least eight times a day, including at night.

Birth control advice

See also the **Family planning** topic of this module for more detail.

Birth control helps families to choose when to have children and how many to have. Waiting at least two years between babies allows a woman to regain her strength after each birth, as well as giving each child the best chance to grow and be healthy.

Even though the woman is still pregnant, she should start to think about methods she can use to avoid getting pregnant again too soon. Her body will need time to recover from the work it endured to grow the baby. For the first six months of the baby’s life, if she is exclusively breastfeeding, she is not likely to get pregnant, but there is still a possibility. The mother and her family should think about ways of planning if and when she will get pregnant again by planning when to have the next baby and how to prevent pregnancy before then.

Sex during pregnancy

It is safe to have sex during pregnancy, and is often desirable for many women. But sex during pregnancy is **not** required for the health of the baby. Some cultures mistakenly believe that semen helps or feeds the growing baby, but this is not correct. Sex during pregnancy should be safe and gentle and not harm the woman or the growing foetus.

Reflection

1. Where do most women in your community give birth? Where do they plan to give birth? At home? In a hospital? Who will assist them to give birth?
 2. Why do parents need a birth plan? How does it help the mother and her family and the health worker be best prepared for any problems?
 3. What are the signs that there may be a problem when a mother should find help? Where will the father or family member go for that help?
 4. Why do we plan for breastfeeding even before the baby comes?
 5. Why is breastfeeding important?
 6. Why should mothers think about planning their next pregnancy while they are still pregnant?
 7. Is semen necessary for the health of the growing baby?
-

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Family planning

Overview

Planning when to get pregnant allows families to be best prepared for growing their families in a healthy and safe way. Different methods of birth control and the supporting services available in the community are provided.

What you will learn

At the end of this topic, you will be better able to:

- tell people about why birth control (contraception) is good for health
- talk to people about healthy timing between pregnancies
- tell people about places in the community that give birth control advice
- list common methods of birth control
- show how to use a condom correctly

Tools

- **Birth control methods** chart
- Which method is best for me?
- **Using male condoms** tool
- **Using female condoms and dental dam** tool
- Male and female condoms for demonstration

Role of the volunteer

eCBHFA volunteers have an important role in talking with the community about the purpose of birth control and managing the timing of pregnancies to keep women and their babies healthy.

Birth control can be a sensitive subject and it is important to learn if there are cultural and religious beliefs or taboos about it in the community. Before advising community members, learn what people do to avoid or plan pregnancy, including whether they use contraceptives. Ask if there are any taboos or special beliefs about preventing pregnancy.

Find out about any birth control services in the community and if everyone can access contraceptives. For example, in some places, contraceptives may only be accessible by married women or for girls and boys over a certain age. Find out:

- Are there any barriers to the use of contraceptives such as lack of access, laws, taboos?
- Do people practise the spacing of pregnancies for when they are ready to expand their family?
- Do women have the power to make decisions about whether and when they get pregnant or not?
- Do young or unmarried people have access to birth control services?

With this information about the community, the subject of birth control can be approached with community members with sensitivity and appropriateness.

Main messages

Learning about birth control and the healthy spacing of pregnancies is good for both the mother and her family. When people have the facts, they can make up their own minds about whether they want to use birth control and if so, when. Both men and women should decide about birth control. The main messages are:

- Birth control is the planning of if and when to have a child. The appropriate timing of pregnancies improves the health of mothers and their children.
- Women should wait at least two years after the birth of their last child or six months after a miscarriage to get pregnant again.
- Pregnancy before the age of 18 increases the health risks for both mother and baby because the girl's body is not yet strong or developed enough to support a healthy pregnancy, putting both the mother and the baby at risk.
- Pregnancy after the age of 35 increases the health risks for both mother and baby. Each year after 35 adds more risk, but a healthy pregnancy is still possible.
- Having more than four pregnancies overall increases the health risks for mothers and their babies.
- There are many safe and effective birth control methods to help people prevent pregnancy.
- Birth control methods can be temporary, long-acting or permanent.
- The only birth control method that protects against pregnancy **and** sexually-transmitted infections is the correct use of condoms.

Reflection

1. Are there women in your community who have had children before they were 18 years of age?
2. Are there women with more than four children?
3. Are there women who have had children after the age of 35?
4. If the answer is yes to any of these, did they experience any problems with their pregnancies or the health of the mothers or babies?

Benefits of birth control and healthy spacing of pregnancies

Birth control gives couples and individuals the choice of:

- when to have children
- how many children to have
- timing between the birth of each child
- when to stop having children

Spacing births at least two years apart helps both mothers and their children stay healthy. Women who space births at least two years apart:

- reduce the chances that their babies will be born too soon or too small
- reduce the chances of problems with the next pregnancy such as anaemia (weak blood), high blood pressure, heavy bleeding after birth or infection
- can breastfeed for two full years
- typically will have more household money to support their children adequately

Breaking down barriers to reliable birth control

More than 214 million women worldwide who are old enough to have babies and who wish to avoid pregnancy are **not** using a reliable birth control method. This is because of:

- limited choice of methods
- limited access to methods, particularly for those who are poor, young, unmarried or disabled
- fear of side effects
- reliance on traditional methods that are often ineffective and sometimes dangerous
- their culture or their religion forbids contraception
- the services that do exist need improving
- women may lack control over the decision to use birth control methods, or their value in the community may be linked to having many babies
- fear of being judged: this is particularly true for adolescents and unmarried people

Volunteers should address these barriers by offering family planning advice in a sensitive but matter-of-fact way. By explaining that use of reliable birth control results in no unwanted pregnancies, families have the choice to decide when and if to have children. Spacing the children at least two years apart is good for the health of both the mother and her children.

In many cultures, there are traditional beliefs that have been passed down for generations about ways to stop unwanted pregnancy. Methods might include tying a rope around your waist to prevent pregnancy, eating certain foods at specific times of the moon cycle, or going through a ceremony with a healer, etc. These methods are not scientifically based or reliable and sometimes can be unsafe for the mother. While a volunteer may personally believe in these traditional methods, remember that the eCBHFA volunteer's role as a community health leader for the Red Cross Red Crescent is to discuss and promote only those methods that are proven to be reliable and safe for women.

Methods of contraception and other advice

Facts about the various methods of contraception (temporary, long-term or permanent) can be taught to community members.

You can use the **Birth control methods chart** and the poster **Which method is best for me?** to show community members the variety of methods of contraception, and what the pros and cons are.

The **Birth control methods chart** outlines the different methods of contraception, providing a description of each. The icons in the last column indicate whether each method is most effective at preventing pregnancy, easiest to use when having sex, prevents sexually transmitted diseases (STDs), makes periods lighter and less painful, requires a visit to the clinic or is an unreliable method.

The poster **Which method is best for me?** gives an overview of the family planning methods and shows more clearly the criteria.

Condoms for STD prevention and protection – Male and female condoms for demonstration, Using male condoms; Using female condoms and dental dams

Condoms are the only effective method to protect both men and women against STDs, including HIV. It is critical that community members know that birth control methods are important for preventing and planning pregnancy, but regardless of the method used, people should ALSO use condoms to prevent STDs and have additional pregnancy prevention.

You can use the two condoms tools to understand and learn about the proper use and disposal of condoms for sex and teach community members how to do the same.

Wrap-up

The volunteers' role is to:

- inform young people about birth control and why delaying pregnancy is healthy. You can work with teachers and schools to reach many young people, both boys and girls, and visit schools to give talks about birth control
- tell community members where they can get advice about birth control
- make sure that everyone in the community is able to access birth control services. This includes women and men who may be on the edge of the community because they are "different", for example, because of their poverty, their race or tribe, their language or their religion
- make sure that young people and unmarried men and women can access birth control services.
- have a supply of Red Cross Red Crescent condoms to offer to community members, and show how they are used
- talk about sexually transmitted infections and how to avoid them

While there are a number of contraceptive methods, not all will be available locally, but many are and often can be found at inexpensive prices or even free at certain clinics or organizations. Volunteers should find out which methods are available in the community and where.

Some birth control methods are temporary. In these cases, the woman or man would have to use a new contraceptive every time they had sex. Some are long-acting and only require a check-up now and then with a health care provider. Some methods are permanent. Male and female condoms, which are temporary methods, protect against sexually-transmitted infections, including HIV/AIDS.

Reflection

1. How does birth control help the mother stay healthy?
 2. How does family planning ensure healthier families?
 3. How much time is best between pregnancies (not just live births) for a mother?
 4. Name two “permanent” birth control methods. How do they work?
 5. Name three “temporary” birth control methods. How do they work?
 6. Which birth control method can protect against sexually-transmitted infections?
-

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Female genital cutting

Overview

This topic addresses what female genital cutting is, why girls and women are cut and who does the cutting as well as the risks and problems associated with cutting and how we can discuss ways to reduce and eliminate harmful cutting practices.

What you will learn

At the end of this topic, you will be better able to:

- define female genital cutting
- explain why female cutting happens
- explain how female cutting harms girls and women
- discuss ways to talk about and potentially end harmful female cutting in your community
- tell girls and women where they can go in your community for help and advice about cutting

Tools

- Doer/non-doer analysis tool results
- Effects of female cutting on women's health
- Reasons why female cutting still exists
- What religion says about female cutting

Topic content

More than 200 million girls and women alive today have undergone female genital cutting (FGM/C) which involves some removal or damaging of female genitals. The reasons for the practice are varied including religious beliefs, traditional practices or beliefs, an attempt to ensure virginity or to ensure fertility, hygiene, or trying to change the way the female genitals look. FGM/C is performed on girls and women of all ages – from newborns to women who have already given birth to a child – depending on the country and the context. There are many short-term and long-term health risks to both the girl and the babies they hope to give birth to when FGM/C is carried out, with issues occurring for many years. The likelihood of girls and women being cut through FGM/C is high in many countries and communities even though it is often against the law and can cause serious health problems, including death. Volunteers can talk and work with a wide group of family and community members to explain the dangers with FGM/C and alternate ways to honour women while also protecting their health and the health of their future children.

Before we begin...

Female genital cutting or mutilation is a sensitive topic for many communities. It is critical that female volunteers properly sensitize the leaders and elders of the community before beginning to discuss this topic. As you begin to teach this topic, engage any elders at the actual training to secure approval.

What is female genital cutting?

Female genital cutting is the ritual cutting, removal or stitching together of a girl's or a woman's external genital tissue. Many girls are cut before the age of five years, although most girls are cut at puberty and some are cut during or after pregnancy. There are different types of cutting and each type has different health risks while no type offers any health benefit – only risk. All types of cutting usually cause health and emotional problems for the affected girls and women throughout adulthood.

There are different types of cutting, some more severe than others. The most common types of cutting is when the clitoris (the female sex organ that is most sensitive, like the man's penis) is cut off. The most severe type of cutting narrows the opening of the vagina and sews the vagina closed by stitching the folds of the genitals. This severe type usually requires cutting and stitching again before a woman can have sex or have a baby. Each time the skin is cut increases the health risks to the woman.

Immediately following FGM/C, the girl or woman who was cut experiences severe pain and heavy bleeding, and can suffer shock, difficulty in passing urine, infections including HIV, loss of fertility, and sometimes death. There are also long-term physical and emotional problems caused by FGM/C.

Reflection

1. Does female cutting happen in your community?
2. If so, at what age are they cut? Who usually does the cutting?
3. What type(s) of cutting are done?

Why do people practise female cutting?

The reasons why female genital cutting is done vary from one community to another and can even vary within families and tribes. The most common reasons people have in support of FGM/C are below. Most of these beliefs are based on incorrect information but may be what people in the community believe. As a eCBHFA volunteer, our job is to address these beliefs. A tool we can use is the *Doer/non-doer analysis* in the *Assessment* module.

1. Some people may believe that FGM/C makes a woman pure because:
 - They believe that cutting ensures that a woman will be a virgin before marriage and will not be able to have sex outside marriage.
 - They believe that a cut woman cannot feel sexual pleasure and is therefore purer.
 - They believe cutting makes a woman more feminine or modest.

2. Some people believe FGM/C is a rite and a necessary part of a community's culture and identity.
 - Some people believe that cutting is a "celebration" of a girl becoming a woman.
 - Some older women who have been cut as girls and who now make their living by cutting younger women promote cutting as a way to continue earning money and to gain importance within the community.
 - Some women see cutting as a way to form a community. Many cut women identify as "members". Uncut women fear they will be excluded by cut women.
 - Some older women may urge families to cut their daughters because this is the way it has always been done and they think it is a matter of honour.
 - Some consider cutting to be a requirement for a girl to get married.
 - Girls who are not cut may suffer discrimination and shame and be excluded from many activities and some rituals in the community¹.
3. Some people believe cutting of a female's genitals to be a mark of good hygiene and beauty. In some cultures:
 - The female sex organs are believed to be dirty, ugly and unfeminine.
 - Girls are considered clean and beautiful only after removal of body parts that are thought to be unclean.
4. Some people believe that cutting is required as part of their religion. However, the sacred texts of Islam, Christianity, Judaism, Buddhism and Hinduism make no mention of female genital cutting and most religious leaders will tell you that that cutting is wrong.
5. Some people make their money from cutting and so will promote cutting as a way to make money using the reasons given above. In some cultures:
 - Midwives deliver babies but also are usually well paid to cut. They encourage women to be cut.
 - Families can expect to receive money and presents from the community when their girl has been cut, so cutting is a way to increase wealth.
 - Cutting may affect the dowry or bride price of a new wife in marriage. A cut girl's family may receive a higher amount of money or goods from the new husband.

Who performs the cutting and when is it carried out?

The cutting is usually carried out by older women, who often have other roles in the community, such as being traditional midwives.

In many places health workers are now carrying out the cutting due to the mistaken belief that it is safer to do it in a medical setting. Community health workers, midwives, nurses and doctors may perform cutting in public or private clinics, in people's homes or even in hospitals. However, no matter who does the cutting or where it is performed, it does not reduce the risks or the harm it causes.

Cutting is mostly carried out on young girls or babies between infancy and age 15, but most often at puberty (when a girl starts her monthly bleeding). Sometimes adult women are cut. It is thought that more than three million girls are at risk of being cut each year.

¹ eCBHFA volunteers can help to teach anti-stigma and anti-discrimination for these girls.

Reflection

1. Who in your community promotes the cutting of girls and women?
2. What are the reasons given in support of cutting?
3. What are some of the concerns the community might have about cutting?
4. Are there any people in your community who do not want to continue cutting girls and women? If so, what are their reasons?
5. What will happen if someone in your community does not support cutting girls?

Based on this discussion, you can start to understand:

- who wants to continue cutting
- how to help develop an accurate perception of the risks following cutting, based on their reasons/behaviours
- who does not want to continue cutting
- how to use their reasons for not wanting to cut (if appropriate) to convince others to stop cutting

You can also begin to explore other rituals that can replace cutting

What are the problems with cutting?

Cutting is not legal in most countries but it is difficult to stop it because people are embarrassed to talk about it and the influence of leaders in the community is strong. While it may be illegal, FGM/C is often done in secret. eCBHFA volunteers can talk with community leaders to help them understand the dangers of cutting and how stopping the practice will result in healthier babies and mothers, and thus a healthier community.

Cutting often results in both immediate and long-term physical, mental and sexual problems such as severe bleeding, problems passing urine, loss of fertility, cysts or unusual growths, infections, problems in childbirth and increased risk of newborn deaths. It can also result directly in the death of the girl or woman who has been cut.

The more tissue that is removed from the woman's genitals, the greater the risks, especially when there is stitching or narrowing of the opening of the vagina.

Immediate problems resulting from FGM/C can include:

- severe pain
- excessive bleeding
- genital tissue swelling and discomfort
- high fever
- infections, like tetanus or HIV
- constantly leaking urine
- cut areas not healing
- shock
- death

Long-term health problems can include:

- infertility or loss of ability to conceive and carry a baby to full term
- increased risk of childbirth problems (difficult delivery, heavy bleeding, Caesarean section, baby not breathing and needing to be revived)
- death of the newborn
- urinary problems (painful to pass urine, infections)
- ongoing problems with the vagina (discharge, itching, other infections)
- menstrual problems (painful periods, difficulty in passing menstrual blood)
- scar tissue and long-term pain
- sexual problems like pain during intercourse
- increased risk of fistula or constant leaking of urine
- increased risk of needing surgery during delivery for safe passage of the baby. If unable to get to clinic in time for the surgery, the baby can die before delivery.
- need for later surgery: for example, the cutting that seals or narrows the opening of the vagina needs to be cut open later to allow for sex and childbirth. This increases the health risks to the woman
- psychological complications or mental problems (depression, anxiety, low self-esteem, post-traumatic stress)

Other long-term consequences of cutting for cut women include discrimination by those who are not cut. Cut and uncut women often will not mix together in a community. Also, increasing numbers of men want to marry a woman whose “rose has all the petals”. Many men state that they prefer women who enjoy sex and cutting decreases the woman’s pleasure.

Reflection

1. What are some immediate problems that a girl or woman might have when she is cut?
2. What are some of the long-term problems of cutting?
3. What makes cutting most dangerous for the girl?
4. How does cutting contribute to fewer pregnancies and more infant deaths?
5. Have you seen or heard about problems from cutting of girls or women in your community? If so, do you believe they are at risk for developing health problems?
6. Do girls, women, boys and men in your community know about the risks when girls and women are cut?
7. What could be the problems in stopping the cutting practice?

Based on these answers, you will:

- know if the community is aware of the risk of women developing health problems
- understand the community’s knowledge of the severity of the potential health problems
- know their perceived barriers to stopping the practice of cutting

Some myths that cut women have about uncut women

Many cultures that practise cutting have many myths around women who are uncut or the hygiene of uncut women. Some myths include:

- having a clitoris causes you to scratch more
- having a clitoris or labia makes you unfeminine
- if you are uncut, you will never be satisfied during sex
- being cut brings you respect

What myths exist in the communities where eCBHFA works?

By talking about cutting and these myths, it takes the secrecy out of the practice and encourages women to talk about these issues and how it affects their health. Only by talking about these misbeliefs and facts about cutting can women be informed decision-makers for their family's health.

Role of the volunteer

Find out what your community thinks about female cutting. Learn who wants to continue the practice of cutting girls and women and what their reasons are. You need to find out why people do not want to stop, so you can understand what the barriers to stopping are. Use the **Doer/non-doer analysis** tool in the **Assessment** module to learn these barriers and identify the best strategy to change their beliefs around FGM/C.

eCBHFA volunteers can share their knowledge about the health problems caused by cutting. Laws in almost every country prohibit FGM/C, resulting in legal fines or even prison for the cutter and the family that allows their daughter to be cut. Many families may not be aware that female cutting is against the law.

Be sure to identify those who encourage and promote cutting, not just the mothers in a community. Mothers are often not in charge of the decision to cut, so involve both parents and even the whole family, as well as leaders in the community. The decision to cut girls is complex. People have different ideas about why they do it, so use the **Doer/non-doer analysis** tool to identify why it still occurs and how to stop the practice. The most likely group to want to carry on cutting are older women who have been cut themselves and see it as a point of honour for the girls and women.

A wide group of community members need to understand the health risks and hear from survivors who have been negatively affected as well as girls who have not been cut and whose lives are better as a result. As part of the change process, volunteers can help girls and their families agree to end cutting and support families who do not wish their daughters to be cut. eCBHFA volunteers can explain that no religious text states or agrees with cutting. Cutting comes from an old slave tradition and has no further place in any community's culture. Cutting is harmful to the long-term health of the girls and women who are cut.

It is important to help the whole community understand that cutting harms not only the girls and women at risk, but affects everyone in the community. Long-term damage can cause loss of fertility, or if they do conceive, problems in childbirth, risk to the lives of unborn children, and possible legal penalties. The income of the family may suffer because they may have to pay for health services for the woman throughout her life.

Learn where girls and women who have been affected by female cutting can go for health services and other help in or around your community. Services may include medical services like surgery and treatment for infections, medical supplies, counselling, etc.

Reflection

1. What are some of the reasons in your community why girls or women are cut?
2. Are you comfortable talking with community members, including men, about the reasons for cutting?
3. Are community members willing to discuss the reasons for cutting?
4. Why do you think cutting is difficult to discuss if it has such serious health risks for women and newborns?
5. If you are a volunteer who has been cut, think about how cutting has affected your health and your family. How can you pass on knowledge about the risks of cutting to others?
6. If you have been cut, how do you feel about teaching this topic? If you are not comfortable, it is important to talk to your eCBHFA supervisor for support or to discuss how another volunteer can support you.

Ways to prevent cutting and alternatives to cutting

eCBHFA volunteers can help communities find safer ways to celebrate girls becoming women. Some communities who want to protect their women and girls are beginning to find ways other than cutting to recognize womanhood.

For example, in some African countries, girls reaching puberty take part in a ceremony that does not involve cutting. This ceremony can last up to one week and provide a coming-of-age celebration for girls and women, complete with gifts, dancing, music and feasting – without cutting. It is a celebration of life and change, where the knowledge and wisdom of older people can be shared with the younger ones.

In Ethiopia, girls have formed “Uncut Girls’ Clubs” to help young women advocate for and lead efforts to end female cutting. In Sierra Leone, volunteers and staff are planning work with School Health Clubs to educate girls about health risks and their rights as children to be protected.

By engaging girls/children – who are most often the victim of cutting – volunteers engage the youth as the strongest behaviour change agents available in a community. When volunteers follow up this kind of intervention with one-on-one household visits with women in a household, the message can be further discussed to shed light on the dangers and stigma of cutting. In countries where FGC is openly discussed, there is a greater likelihood of reducing cutting because the issue is examined from a health standpoint and shown the lack of any health benefits gained from cutting.

Involving men and boys is vital to help change attitudes towards female cutting. Programmes in some countries are holding “community conversations” with villagers every two weeks with the help of trained leaders to try to reduce cutting. By involving men and boys in these advocacy efforts, there is a better chance to change social norms and shift the thinking around FGM/C towards healthier behaviours around girls reaching puberty.

Reflection

1. What are some facts to teach people about the dangers of cutting? What might help people understand the dangers, both short- and long-term?
2. What would happen in your community if a family decided not to cut their daughter?
3. Are there any traditions that your community once had, but have now stopped? If so, why and how?
4. What are the advantages of not being cut? What are the disadvantages of not being cut?
5. How can your community celebrate the move from girlhood to womanhood without cutting?

What to do if the community wants to continue cutting

Like other behaviours, people will only change their behaviour when they are at the approval and intention stages of behaviour change. By identifying Doers, or girls' families who did not subject them to cutting, eCBHFA teams can discover how to learn from these Doers and apply this information to understand how to address the issues that support cutting.

If a community remains resistant, work with the community leaders and families to then agree that girls should not be cut when they are under age as this is a child rights violation. If a girl reaches the age of consent, she then is allowed to make an informed decision about her own body. Most girls and women who are being cut against their will are victims of child rights violations. If a community practises the behaviour, encourage that they wait until the child is of legal age and can make her own decision and choices.

The Red Cross Red Crescent role in cutting

Cutting is a serious health risk and one that is not aligned with the Red Cross Red Crescent mandate of Humanity. Red Cross Red Crescent medical professionals, health workers or volunteers that either directly perform FGC or contribute materials to conduct FGC violate the mandate of Do No Harm. Cutting has no health benefits and thus, the Red Cross Red Crescent view on cutting is that no staff or volunteer should perform or contribute to FGC. It is a violation of the principle of Humanity.

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Menstrual hygiene

Overview

Approaching the sensitive and often taboo subject of menstruation (a girl or woman's monthly bleeding or period); giving the facts about menstruation to girls and women, boys and men; helping the community to provide safe and hygienic places for women and girls when they have their period.

What you will learn

At the end of this topic, you will be better able to:

- give the basic facts about menstruation
- say why hygiene is so important when women have their period
- give girls, women, boys and men facts about good menstrual hygiene
- promote mutual understanding and respect between girls, women, boys and men
- help the community to find ways to improve menstrual hygiene, such as improving toilets and washing facilities and helping to make sanitary protection materials (pads, cloths, etc.) available and inexpensive
- identify where girls and women can go in the community to get help on menstrual hygiene issues

Tools

- **Key messages about menstruation** poster
- **Use and disposal of your pads** posters
- **Use and care of your reusable pad** posters

The role of the volunteer

Menstruation is often a taboo subject, with strong cultural or traditional beliefs. This often has an impact on the life of women when they are having their period. For example, they may not be allowed to take part in religious events. In many cultures they are considered "impure" or "dirty" when they are menstruating and may be excluded from the community when they are bleeding. Women and girls often feel shame or fear that everyone will know when they have their period. You need to know what the beliefs in the community are, so that you can approach the subject in a sensitive way.

Because of the taboo, it is a difficult subject. You can tell women and girls that menstruation is a normal bodily function and not something to be ashamed of. Tell girls they should be proud that they are becoming women. Tell them the facts about the purpose of menstruation and what happens.

You can teach about menstrual hygiene and can help girls and women cope with menstruation safely, in privacy and with dignity. You can also teach boys and men the facts about menstruation.

Key messages – Key messages about menstruation poster

The key messages to pass on about menstruation are:

- Women have a menstrual cycle which is usually around a month. This cycle helps their bodies prepare for pregnancy. It is a normal bodily function for women.
- The period is the monthly flow of blood from the vagina when your body sheds the lining of the womb which is not needed if you are not pregnant.
- Menstruation is an indication that a girl is becoming a woman (in terms of being fertile) and usually starts between 10 to 15 years of age.
- Women usually stop having their periods between 45 to 55 years of age.
- Under normal circumstances, most women get their period approximately every 28 days and the bleeding lasts for five to seven days, although the number of days between each period and the amount of bleeding varies from woman to woman, and especially when a girl is just beginning to get her period.
- Some women feel cramps (pains in the lower stomach) just before and during menstruation, and especially on the first day. The cramps are normal. They are caused by contractions of the womb as it sheds the lining (the menstrual blood).
- Some girls and women also experience pain or soreness in their breasts and back pain during menstruation.
- Bathing when you have your period is safe.
- Having sex is safe during menstruation, but it depends on the culture whether it is taboo or not.

What is menstruation?

Menstruation is a natural and normal part of every girl's and woman's life. Menstruation – also called a woman's period – is the female body's way of clearing the uterus of blood that is no longer needed to support a pregnancy as there is currently no fertilized egg growing in the woman's body. It is normal and natural and a regular and necessary part of how the female body works. Menstruation does not mean that a girl or woman is unclean – it is exactly the opposite as her body is taking care of itself to prepare for the next cycle of menstruation. Some communities may make girls or women feel badly or dirty for menstruating, but eCBHFA volunteers should discourage this and explain that it is important to support girls and women who are menstruating – not shun or be cruel to them. Every person in the world was developed as a result of a functional menstrual cycle and we should respect this natural and normal function.

Should only girls and women to know about menstruation?

All community members need to know about good menstrual hygiene, not just the girls and women. Men and boys also need to know the facts and be encouraged to support women and girls during menstruation. Ask boys and men to think how they could support their wives, daughters or sisters when they are having their period. Make sure that boys and men understand the link between menstruation and the ability of the girls and women to have babies. Knowing the facts helps to reduce stigma and helps lead boys and men to respect girls and women.

With a mix of community members, you can discuss ways in which the community can support women and girls to manage menstruation with dignity.

Cleaning the vagina

Menstruation is one of the ways that the female body cleans the uterus and vagina. The vagina does not need to be additionally cleaned during or after menstruation, as menstruation is the method of cleaning.

The community may have incorrect information about menstruation. Talk with the men and women separately to understand their beliefs and correct misinformation. Some people incorrectly believe that semen cleans the vagina. Some incorrectly believe that douching or using your finger to clean the vagina is necessary.

Putting anything into the vagina to clean it can cause harm or even infection. A woman's body is very intelligent and the vagina is self-cleaning – the only cleaning needed is to use a gentle soap with water to clean the outside of the vagina.

Good menstrual hygiene management – Reusable pads posters; Disposable pads posters; Use and disposal of your pads poster; Use and care of your reusable pad poster

Good menstrual hygiene involves the girl or woman keeping her body clean and managing and disposing of the menstrual blood in an appropriate way. When girls and women exercise good hygiene menstruation, they are keeping their bodies healthy. When boys, men and other women support the girls or woman to keep good menstrual hygiene, it makes for healthier and happier families. When women are forced to physically separate from their family during their period, girls and woman are negatively affected and can even result in the girl or woman finding themselves in unsafe situations that may even result in death. Support girls and women.

The key messages to give women and girls about good practice in menstrual hygiene management are:

- to wash the skin **outside and around** their vagina at least once a day during menstruation
- do not wash inside the vagina or douche because it can cause an infection. The vagina will clean itself – no need for douches or other types of cleansers.
- do not insert items or fluids into the vagina to clean it. Semen does not clean a vagina. Vaginas clean themselves.
- always use a clean and dry cloth or pad, a new tampon or a clean menstrual cup as sanitary protection
- do not share sanitary protection with others
- change menstrual cloth or pad (both disposable and reusable) and tampons three to six times a day, depending on the flow
- if using a menstrual cup, empty and wash every four to eight hours
- if you use dirty or damp cloths and pads, you can have itching or infection
- wash hands after changing your sanitary protection to remove any blood or germs on your hands
- wash reusable menstrual cloths or pads with soap and water and dry in the sun, if possible

There are many different menstrual hygiene products available for women. These include reusable pads, disposable pads, tampons and menstrual cups.

Any product used should be changed regularly to keep the area clean and less likely to cause bad smells or infection.

It is important to correctly dispose of pads or cloths that are not going to be washed. These should be placed in a rubbish bin, bucket or pit, and NOT in a toilet or latrine.

If you plan to reuse the reusable pad, wrap the reusable cloths or pads in a leak-proof bag until they can be washed and dried properly. Do not attempt to reuse disposable pads.

You can refer to the **Use and disposal of your pads** poster and **Use and care of your reusable pad** poster to support women to exercise good menstrual hygiene.

What are the health risks due to poor menstrual hygiene?

Poor menstrual hygiene increases the risks of developing other health issues, including:

- rashes
- vaginal yeast infections
- urinary/reproductive tract infections
- bad odours

Improving menstrual hygiene in the community

Both girls and boys should be taught about menstruation and menstrual hygiene. Many schools have health education or life skills classes. You can work with teachers to ensure that both girls and boys know the facts about menstruation and how the menstrual cycle is part of the cycle that enables women to grow babies in their bodies. Girls and women should be taught early, preferably before menstruation, about what menstruation is and how to care for their bodies during this time. This preparation helps both girls and boys to understand and respect what is happening when a girl or women experiences menstruation.

In the community, leaders and members need to know about the importance of menstrual hygiene. Work with community members to identify some of the main barriers to good menstrual hygiene. Clean, safe and private latrines, enough clean water and washing facilities and rubbish bins for disposal which are emptied often are needed. Work with the community to identify ways to make sanitary protection materials easily available at a lower cost.

A support group in the community can help people to talk about problems with menstrual hygiene and to find ways of improving the situation. Existing women's groups are good starting points. Issues such as where to get sanitary goods may be important, for example, if there is stigma about buying the products in a public place. In this case, health centres or health workers could be asked to keep a supply of the products.

Reflection

1. Are there any taboos in your community about menstruation? If so, how do these taboos affect girls and women?
2. What changes in a woman or a girl's days when she has her period compared to when she does not have it? Are there things she is not allowed to do? If so, what are they?
3. Do people in your community (men, women, girls and boys) know basic facts about menstruation?
4. Where do girls and women get information about menstruation? Who can they trust to talk about it?
5. What do women and girls do in your community to maintain good menstrual hygiene?
6. Are there safe, clean and private places for girls and women to change, dispose of, or wash and dry (if using reusable) cloths and pads in your community?
7. Do schools have private latrines where girls can feel safe when they are having their period?
8. Are menstrual supplies (sanitary protection) available and inexpensive in your community? Can everyone get them without stigma or discrimination, especially those who may have disabilities?
9. Is there adequate water in places where women change their sanitary protection to wash their hands and bathe during menstruation?
10. Do men and women (boys and girls) know about the risks ensuing from bad menstrual hygiene?
11. What beliefs or misunderstandings do people in your community have about menstruation or how to clean the vagina? How will you correct false beliefs?

Further information and support

Where can girls and women go for information about menstrual hygiene? You can work with the Water, Sanitation and Hygiene (WASH) team at your local Red Cross Red Crescent branch to find ways of helping the community to have safe and clean water, latrines and education. You can also refer to the IFRC's menstrual hygiene materials available at IFRC's WASH resource page: <http://watsanmissionassistant.org/> or on IFRC's Learning Platform at <https://ifrc.csod.com/phnx/driver.aspx?routename=Social/Communities/LearningCommunityWithFeed&Root=25> for more information.

Work with other eCBHFA volunteers to find out where girls and women can go to get information and practical help on menstrual hygiene.

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Newborn and new mother care

Overview

Advice for a new mother and her family on the care of her newborn; care for the new mother; what to look out for that may mean there is a problem with either the mother or the baby after the birth; how to prevent infection; good healthy food; emotional support for the mother.

What you will learn

At the end of this topic, you will be better able to:

- show the mother and her family how to care for both mother and baby after the birth
- describe "kangaroo mother care" and why it is good for baby and mother
- list the signs that may mean there is a problem with the mother after the birth and what to do if any of the signs are seen
- list the signs that may mean there is a problem with the newborn after birth and what to do if any of the signs are seen
- describe how to protect the newborn from infection
- advise mothers and their families about the importance of healthy food and rest
- advise about birth control after the birth

Topic tools

- Kangaroo mother care poster
- Signs that mother is having a problem after birth poster
- Ways to keep newborns safe poster
- Two handfuls of signs poster

Role of the volunteer

Even if it is a mother's second baby or she has given birth to many babies, there may still be something she does not know that can help her and her family. eCBHFA volunteers can play an important role in helping the mother and her newborn be healthy after the birth by telling the mother and her family about:

- caring for the mother and the newborn
- why the baby should be given only breast milk for the first six months (see the Breastfeeding topic for more information)
- signs that there may be a problem with a mother or her newborn and what to do if any of the signs are seen
- telling the mother that both she and her baby should visit a health worker or clinic after birth for a health check
- getting the baby immunized against certain diseases
- birth control and family planning

Volunteers should include as many family members as possible (fathers, grandmothers, aunts, sisters, even brothers) when giving advice to the mother and give the mother and her family time to ask questions.

Health visits after the birth

The mother and newborn should receive a visit the day after the birth and more visits later from a health worker who will check the mother and baby. The health worker or volunteer will:

- look for any signs that there may be a problem
- check mother and baby for any signs of infection
- take the mother's blood pressure
- ask her about breastfeeding and any pain in the breasts
- ask her about any difficulties going to the toilet
- ask about any pain from tearing
- ask about headaches
- ask how she is feeling emotionally

The health worker or volunteer will check the newborn for signs that there may be a problem, watch while the mother breastfeeds, check the umbilical cord (the cord attached to the baby's belly button), ask the mother if the baby has passed urine and stool, and talk about immunizations.

Checking the mother after she has given birth

Birth can be a difficult and scary experience and many new mothers need time to recover. Volunteers are advised to check in on the new mother as soon as possible after the birth.

Volunteers should wash their hands carefully before touching the mother or baby, or at least in the first 24 hours after birth. Babies have very limited immunity when they are first born and are very susceptible to sickness, so all visitors should help keep the baby safe by washing their hands or limiting contact.

With washed hands:

- gently but firmly rub the mother's belly right below her belly button. Her belly should feel firm (like a large grapefruit)
- there should only be a few clots of blood from the vagina, not a steady flow

Volunteers or the family should call for help if they think the mother is bleeding too much and her belly is not firm. Breastfeeding the newborn helps to make her belly firm and makes her bleed less after the birth, so advise the mother to breastfeed exclusively. Call for help if the mother's vagina or other areas have been torn from the birth. They may need stitching.

Caring for the newborn immediately after the birth

Newborns lose heat quickly after birth. If a newborn becomes cold, it can lead to problems with breathing and infection. Being wet, being placed on cold surfaces or being in a cold room are dangerous for newborns. The baby should not be bathed for 24 hours from birth, when the body temperature becomes more stable.

The baby's body, including the head, should be dried immediately with a clean, warm cloth. As well as stopping the baby from becoming cold, this helps to make the baby breathe. The baby should be a healthy colour and feel warm. Check to make sure the baby is breathing.

Once the baby is dry, the baby should be placed between the mother's breasts and covered with a dry cloth. The baby's head should be covered with a cloth or a hat.

Mother and baby kangaroo care – Kangaroo mother care poster

"Kangaroo mother care" is a good method of care for new babies. Just like a kangaroo with her infant in the pouch, the newborn should have immediate skin-to-skin contact. Usually, this will be with its mother but it can also be the father or other family member. Skin-to-skin contact is perfect for keeping the baby warm, as well as making a bond between the baby and the mother. It makes the baby want to breastfeed and helps protect it from infection. Kangaroo care also allows the mother to easily breastfeed the baby when needed, make eye contact, ensure their baby is well and breathing easily.

Volunteers can help place the baby in the right position for "kangaroo care" and demonstrate the ease of this position. Demonstrate how to do so using a large clean cloth to keep the newborn in place and help the mother to put the baby in the right place:

- Place the baby in an upright position between the mother's breasts, chest-to-chest.
- Position the baby's hips in a "frog-leg" position with the arms also bent.
- The baby's belly should be somewhere at the level of the mother's belly.
- The mother's breathing helps the baby breathe.
- Make sure the baby stays in this position by using the clean cloth to gently bind the baby in place.
- Turn the baby's head to one side so that it can breathe and make eye contact with the mother.
- Tie the cloth firmly with the top of the cloth just beneath the baby's ear.

Reflection

1. Why is it bad for newborns to become cold?
2. What is the best way to keep a newborn warm?
3. Why is skin-to-skin contact good?
4. Do people in the community know the steps to caring for new mothers and their babies after birth? How can we help people apply them?

Check for signs that there may be a problem with mother and baby – Signs that mother is having a problem after birth poster; Ways to keep newborns safe poster

Teach the mother and her family members the signs to look out for after birth that may mean there is a problem with the mother. Tell them that they must ask for help straight away from a health care worker if they see any of the signs. These are:

- heavy bleeding (soaks a pad or cloth in less than five minutes)
- bleeding that smells bad
- severe pain in the belly (different from the cramping some women feel when they first breastfeed the baby)
- pain in the chest or legs
- severe headache or trouble seeing
- fast breathing or difficulty with breathing
- fever
- chills
- vomiting
- fits (shaking that does not stop)
- too weak to get out of bed
- unable to pass urine or leaking urine
- swollen, red or very tender breasts (especially combined with a fever)

For the baby, the first handful of signs that there may be a problem are:

- Low or high temperature: if fever, the baby feels hot and has a high **temperature**. The baby's mouth will feel hot on the nipple during breastfeeding. If low temperature, the baby is cool or cold to the touch.
- Rapid and difficult **breathing**: The baby pants like a dog or breathes more than 60 breaths per minute.
- Weak **cry** (the baby makes a low groaning sound "eeeh, eeeh").
- Little or no **activity**: The baby has abnormally pale skin or does not respond to sound or touch or the baby is limp.
- Weak **sucking**: The baby only weakly sucks on the breast or stops sucking on the breast or stops wanting to breastfeed.

The second handful of signs that there may be a problem are:

- chest becomes **sucked** in (in-drawing) when the baby breathes
- jaundice in the first 24 hours of life (the baby's skin looks **yellow**), or yellow palms and soles of the feet at any age
- **convulsions** (the baby does not stop shaking)
- redness of the skin, swelling, pus or bad odour around the cord or **belly button**
- red, swollen eyelids and pus discharge from the **eyes**

Newborns that die often die unnecessarily because the signs that there may be a problem are not recognized. Once the mother or families see any one of the danger signs, there should be no delay in getting help or medical care.

An easy way to teach mothers and family members to remember the most important signs that there may be a problem with a newborn is to use your hands, with each finger and thumb in place of a sign. Be expressive when you show them. For example, mimic fast breathing, a distressed cry, and a floppy baby that does not respond to the voice or touch. Show the chest sucked in, shake or tremble to show convulsions or mime sore eyes. Prompt community members to use their hands and repeat the signs back. Children in the family can learn the signs too.

Reflection

1. Do people in your community recognize the two handfuls of signs that there may be a problem with the newborn?
2. Do you know of any cases where newborns have died because these signs have not been recognized?

Protecting the newborn from infection – **Ways to keep newborns safe poster**

Just as volunteers must remember to wash their hands with soap and water before and after touching the newborn, the mother should not allow too many people to handle the baby in their first few weeks of life as they have weak immunity at first. Anyone who does touch the baby should wash their hands before and after. Keep sick children and adults away from the baby.

The new mother should give the baby only breast milk because it helps protect the baby from infection.

Immunizations

After approximately 3 weeks old, babies should start receiving appropriate vaccinations based on the timetable outlined both by the Ministry of Health or accessible in the eCBHFA **Immunization** module.

Babies should be vaccinated against some of the most preventable but dangerous diseases. Each country does this in its own way, but all Ministries of Health have a plan for vaccinating babies.

Learn the vaccination schedule in your country and tell the new mother to contact the health centre/clinic/hospital to obtain the vaccinations for her baby. In many countries, the typical vaccinations for newborns include:

- BCG to prevent tuberculosis, but this is not given if the baby is HIV-positive
- Hepatitis B
- Polio

For more information on immunizations needed to keep newborns and children safe from preventable diseases, refer to the **Immunization** module of eCBHFA.

The umbilical cord

Make sure that anything that touches the baby, like blankets or bedding, is clean and dry. It is also important to keep the umbilical cord, which is the cord attached to the belly button, clean and dry until it finally falls away. The health worker or midwife who helped at the birth should have cut and tied off the cord once it stopped pulsing after the birth. Nothing should be put on the cord and it should fall off between five and ten days after birth. It should be checked every day to check for any sign of infection. The signs of infection are:

- cord not dropping off ten days after birth
- discharge with pus
- bad smell
- redness and swelling of the skin around the cord

The mother should get medical help immediately if any of these signs are seen.

Eye care

Explain to the mother that after breastfeeding for the first time, a health worker should give the newborn eye care to stop infection. The health worker will put special drops or ointment into her baby's eyes.

Vitamin K

Tell the mother that a health worker should also give a Vitamin K injection to the baby. This is usually given to newborns within the first few hours after birth, or as soon as possible, to prevent bleeding.

Weighing the newborn

A healthy weight is a good indicator of a healthy baby. All newborns should be weighed at birth after their first breastfeeding or as soon as possible within the first 24 hours. Newborns that have a low birth weight (less than 2,500 grams) are especially at risk and need kangaroo mother care, more frequent breastfeeding and extra care and attention.

In areas where there is malaria

Remind the mother that every night, she and the baby should sleep under a bed net that has been treated to kill insects. She should use the bed net every night to avoid getting bitten by the mosquito that carries malaria. It is a disease that is very dangerous for both mother and baby.

Emotional support

Remind the mother that any time someone is sleeping during the daytime such as for naps or early morning, she and the baby should sleep under a bed net that has been treated to kill insects. She should use the bed net as much as possible during the day – and especially when napping during the day – to avoid getting bitten by the mosquito that carries dengue, chikungunya or Zika. They are diseases that are very dangerous for both mother and baby.

Reflection

1. What are the most important ways to protect the newborn from infection?

Keeping mothers healthy

Emotional support

Tell the new mother that she needs to eat a greater amount and variety of healthy foods to help her regain strength after giving birth. Eating well also helps her to produce enough breast milk. Advise the mother and her family that she should eat foods such as meat, fish, nuts, seeds, cereals, beans, vegetables, cheese and milk. New mothers should avoid too much sugar or alcohol as any food that she eats will also be taken in by the new baby through breastfeeding.

You should also:

- discuss any taboos that exist about foods that are healthy and give accurate information. For example, there may be wrong beliefs that some fruits and vegetables affect milk production, or that eggs or fish will affect the baby's appearance, or that mothers should only eat cool or hot foods
- advise the mother and her family that she should rest to recover from the birth
- advise the mother she should take iron and folic acid supplements for three months after the birth
- advise the mother to get to the clinic to check her body within one to two weeks of giving birth to ensure she is healing well

Remember that all the healthy food that the baby requires comes from breast milk. Remind the new mother to give the baby only breast milk for the first six months (see the topic: **Breastfeeding**).

Emotional support

A new mother can feel worried and anxious about caring for a newborn baby, especially if it is her first child. During visits, ask the mother about her emotions and feelings. Tell her that it is normal to have low energy, feel tired and even feel especially emotional, particularly after a long labour or a difficult birth. Praise her for giving good care to her newborn. Talk to her family members about how to give the new mother emotional support. Other members of the family can be taught how to hold the baby to give the mother a rest when she needs it.

Birth control

Talk to the mother, and father if available, about why it is good not to get pregnant again too soon. After a live birth, it is best for the mother to give her body rest for approximately two years before trying to get pregnant again. This length of time gives the new baby the best chance to grow and be healthy and helps the mother's body to grow strong again to support the next baby.

Remind her that giving the baby breast milk alone for six months means that she may have some birth control protection during that time, but also discuss other methods of birth control (see the **Family planning** topic). Birth control allows for sex without getting pregnant before your body and family are ready to have another baby.

Reflection

1. Where can women in your community go for help and support after giving birth?
2. Do new mothers and newborns often have problems in your community? If so, what are they? How could you help with these problems?
3. Which ways can eCBHFA volunteers support care for new mothers after giving birth?
4. Why is it important to wait two years before becoming pregnant again?

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Post-abortion care

Overview

What abortion is; giving help to a woman who has had an abortion, whether sudden or planned; what to expect; what are the signs that there may be a problem; caring for herself; helping with her feelings by being understanding and not judging.

What you will learn

At the end of this topic, you will be better able to:

- support a woman who has had either a sudden or planned abortion
- list the signs in a woman after an abortion that may mean there is a problem
- advise the woman about caring for herself and what to expect after an abortion
- tell the woman where to get help in the community if she has any problems or sees any of the signs that may mean there is a problem

Tools

- Post-abortion care poster
- Psychological first aid topic from the Psychological and basic first aid module of eCBHFA

What is abortion?

Abortion is the termination of a pregnancy, either naturally or planned. Abortion means ending a pregnancy before the unborn child can live on its own outside its mother.

More than one in ten pregnancies are terminated by a type of abortion. Abortions that happen suddenly and unexpectedly before 24 weeks of pregnancy are called a miscarriage. Miscarriages are most common in the first 12 weeks of pregnancy. An abortion that is planned is called an induced abortion and is usually done to end an unwanted pregnancy.

Whether the pregnancy was wanted or unwanted, the time after an abortion can be difficult for the woman emotionally.

Role of the volunteer

You can play an important role in helping a woman recover, both with her body and with her feelings after an abortion by giving advice and being understanding and supportive to her and her family. Whether an abortion is planned or not, a woman often has feelings of guilt and loss. You can help her to share her feelings and worries and tell her that it is normal to have feelings of sadness and loss and to feel low. Suggest that she talks to a trusted health worker, family member or friend. If she is depressed and needs professional assistance, refer her for help.

It is strongly advised that the eCBHFA volunteer provide psychological first aid (PFA) to the woman and some family members who may be in distress over the loss of the pregnancy, even for planned abortions. Refer to the **Psychological first aid** topic in the **Psychological and basic first aid** module of eCBHFA for detailed guidance.

If the abortion was planned, you must remember the Red Cross Red Crescent principles, and always respect and not judge her decision.

Changes to the body after an abortion

Tell the woman that she can expect some changes in her body after an abortion. These changes include:

- swollen breasts
- light bleeding from the vagina or spotting for a few days to a few weeks
- mild belly cramps
- back pain
- some weight gain
- some vaginal discharge
- menstrual period or monthly bleeding may return in a few weeks or take longer in some women

Caring for herself after an abortion

Talk to the woman about looking after herself after the abortion. She should:

- use clean cloths, pads or cotton wool for the light bleeding and change them every two to four hours
- drink plenty of water, tea, juice or other fluids
- eat healthy foods, especially foods that are rich in iron, such as meat, fish, eggs, pulses and beans, nuts and seeds, dried fruit and whole grains such as brown rice
- get plenty of rest and avoid lifting anything heavier than four to five kilos or over her head for at least two to three days
- return to her normal life only when she feels better
- avoid sex or putting anything in the vagina for at least five days after the abortion and three days after the bleeding has stopped

Even though a normal menstrual period usually only returns within four to eight weeks, that timing can vary for each woman and each abortion. Even though her menstruation may not have returned, a woman is at risk of getting pregnant again within ten days of the abortion. When she decides to have sex again, she should use a birth control method (see the **Family planning** topic for more information), even if she suffered a miscarriage and wants a baby. If a woman wants to get pregnant again, it is strongly advised that she waits for at least six months to allow her body to recover and to have a healthier pregnancy next time. By waiting these six months, her body will be stronger for the next pregnancy.

Signs that there may be a problem after an abortion

Tell the woman about the signs that mean she must ask for help from a health worker if she sees any of them. The trouble signs are:

- heavy bleeding from the vagina (more than a normal menstrual period)
- bleeding that lasts more than two weeks after the abortion
- constant cramping and severe pain
- fever or chills
- smelly liquid coming from the vagina
- vomiting or feeling sick
- confusion or fainting
- paler skin than normal
- pain or redness in the breasts. If the abortion occurred late in the pregnancy while the woman was starting to make milk for the baby, their breasts may become engorged and infected

Finding help

If a woman shows any of the signs above, she must see a health worker. There is a risk of excessive bleeding or even death. Tell her where she can go in the community for medical help if she has problems or sees signs that there may be a problem.

In some countries or places, induced abortion may be against the law. Even if abortion is against the law where you live, if someone has any of the problems above, they should get to a doctor or clinic immediately or they risk serious illness or even death from problems associated with the abortion.

Reflection

1. Is planned (induced) abortion legal and available in your community?
2. Where do women go in your community to get an induced abortion?
3. If induced abortion is legal in your community, why do some women still seek abortion care from unqualified providers?
4. Do you know women in your community who have been forced to get an abortion (by their husband, family...)?
5. Do you know women in your community who have been forced to give birth (by their husband, family...) even if they were unwilling?
6. If induced abortion is not legal in your community, what do women do when a pregnancy is unwanted?
7. Where can a woman go in your community for advice about her pregnancy?
8. What are some of the changes in her body a woman can expect after an abortion?
9. What are some of the signs that there may be a problem after an abortion?

eCBHFA Volunteer manual

Module: Maternal and Newborn Child Health

Topic: Preparing for safe birth

Overview

Helping pregnant women and their families to plan for the baby to be born at a clinic or hospital; advising them on what to do when labour begins; showing the family how to support the woman during labour, birth and afterwards; signs in the newborn that there may be a problem.

What you will learn

At the end of this topic, you will be better able to:

- advise a pregnant woman and her family about getting ready for a safe birth
- list the early signs of labour
- tell the family how to support a woman during labour, delivery and immediately after the birth
- describe signs that there may be a problem with the newborn
- know who the mother should contact in the community for care and support if there are problems

Tools

- **Birth plan checklist** poster
- **Baby is coming! Early signs of labour** poster
- **Two handfuls of signs** (that there may be problems with the newborn) poster

Role of the volunteer

You play an important role in giving advice and support to pregnant women and their families. Tell mothers that the safest place to give birth is in a clinic or hospital. They should follow the birth plan that they made with their family. This plan will include what to do if there is a problem. Remind the woman and her family about the signs that there may be a problem during pregnancy and what they plan to do if they see any of the signs (see **Care during pregnancy** topic for more information).

Volunteers will give information to the family about labour as the time when the woman is about to give birth, what happens and what the woman should do. Family members can learn how to help the woman during labour, birth and during the days after the birth.

Volunteers can also prepare the mother and family before the baby is born about the signs that there may be a problem with the newborn and how to get help immediately if they see any of the signs.

Home delivery

Many women want to give birth to their baby at home. They may feel more comfortable there, or they may live far from a hospital, or they have other children they need to look out for and cannot be away from home. Regardless of the reason, it is best to counsel mothers – especially new mothers – to consider giving birth in the hospital. This is for the safety of the mother and the child as complications may arise during or after delivery. While traditional birth attendants can manage routine births, it is not always possible to know that a problem can arise – until it is too late to get to the hospital.

Complications during and after labour and delivery can endanger the health or life of the baby or the mother, or both. If there are no health workers present at the birth, the time it takes to get them to the home in an emergency may be longer than necessary to save the baby or mother's life if something goes wrong.

Advise mothers to give birth in hospital when at all feasible.

Birth plan – Birth plan checklist

By developing a birth plan, a mother is prompted about the various decisions and options she has when it is finally time to give birth. By helping the mother consider her different options, this gives her the feeling of more control over delivery, especially for mothers who are anxious about giving birth. Labour and delivery are a time when a woman may not feel able or comfortable making serious decisions, especially if there are complications.

The **Birth plan checklist** poster starts at the beginning of the path with the prompt "When is the baby due?". You have seen in the **Care during pregnancy** topic that knowledge of when the baby is due to come can help an eCBHFA volunteer to advise the mother to plan her health checks, help her monitor her weight and the movements of the baby. Closer to the birth date, you will want to check in more regularly with the mother to make sure that all is proceeding to plan. The birth plan has been recorded and you can revisit the questions along the path in the **Birth plan checklist** poster from time to time to check that nothing has changed.

When labour begins – Baby is coming! Early signs of labour poster

You can advise a woman that when she begins to notice early signs of labour, she should remain calm and alert the family or a friend, so that they can help prepare for the birth as in the birth plan they made together.

Early signs of labour are:

- clear or pink-coloured liquid comes out of the vagina
- a mucous plug comes out of the vagina
- clear water comes out of the vagina
- bright red blood comes out of the vagina
- low, dull backache
- pressure at the base of the belly
- vomiting
- pains, contractions (when the muscles around the belly tighten and relax) or cramps with the entire pregnant belly becoming hard for at least 30 seconds at least five times in one hour

In early labour, the woman should:

- take slow, deep breaths during the contractions
- relax her body between contractions
- drink plenty of sweet liquids and/or tea
- take honey, if available, for energy and for soothing the throat
- pass urine often
- eat a little if hungry
- make sure all the things needed for the birth are ready (pads, clean clothes for the mother and baby, clean and dry cloths, blankets, a hat for the newborn, soap, money for expenses)

When the contractions are five to ten minutes apart and lasting at least 30 seconds to a minute each, it is time to go to the clinic or hospital or to contact the trained birth attendant, depending on the birth plan.

Help during labour, delivery and immediately afterwards – Two handfults of signs poster

All mothers should get help and care during labour, the delivery and immediately after the birth.

Teach the family to give this help and care. They should:

- offer fluids to the woman in labour, such as water, juice or tea with honey. Honey can be given by the spoonful. It helps to give strength to the woman and soothes the throat
- get her to change position frequently. Help her to walk around, lie on her side or sit in a chair. This helps labour to progress
- rub her back and shoulders, hold her hand and breathe with her during the contractions. You can show her how to breathe in through her nose and breathe out with a long slow breath while making a “shhh” sound during each contraction. It helps her to relax during labour pains
- wipe her face and neck often with a clean, cool, wet cloth. Labour is hard work!
- say comforting things. Tell the woman that every contraction brings her closer to having the baby. She can do it. Labour will not last for ever. Praise her for getting through each contraction
- get the woman to sit upright to push and deliver the baby. Put cushions or blankets behind her back (or someone can sit behind her and hold her). This helps deliver the baby more easily and avoids tearing between her legs
- put the newborn between her breasts immediately. This allows the baby to feel for the breast and start feeding. The baby should breastfeed within the first hour after birth. All the time, there should be skin-to-skin contact with the mother. Breastfeeding immediately is good for the newborn and the mother. It helps the mother’s belly to shrink, prevents too much bleeding and helps the milk to come. See the Breastfeeding topic for more on helping the mother to learn to breastfeed

- teach the mother and her family to rub her belly frequently after the placenta (the afterbirth) is delivered. This helps her belly to remain firm and prevents too much bleeding. The belly should feel like a firm grapefruit below her belly button
- get the new mother to drink frequently after the birth (water with lemon, juices, broth, tea with honey or sugar). This helps the mother's body to recover from the birth and to produce breast milk
- teach the mother and her family the "two handfuls" of signs that there may be a problem with the baby (there are ten of them, one for each finger and thumb on the hands). If they see any of the signs that there may be a problem, tell them to get help immediately from a trained health worker. See also Care of the newborn and new mother topic.

The first handful of signs that there may be a problem with the newborn:

1. Rapid and difficult **breathing** (the baby pants like a dog)
2. Weak **cry** (the baby makes a low groaning sound "eeeh, eeeh")
3. Fever (the baby feels hot and has a high **temperature**)
4. Weak **sucking**
5. Not very **active** (floppy, **pale** skin, does not respond to sound or touch)

The second handful:

1. chest becomes **sucked** in (in-drawing) when the baby breathes
2. jaundice in the first 24 hours of life (the baby's skin looks **yellow**), or yellow palms and soles of the feet at any age
3. **Convulsions** (the baby does not stop shaking)
4. redness of the skin, swelling, pus or bad odour around the cord or **belly button**
5. red, swollen eyelids and pus discharge from the **eyes**

Reflection

1. Do women in your community know most or all steps to getting ready for a safe birth? Do they apply them?
2. Do they experience any problems during pregnancy? If so, what are they? How can you help?
3. Do people (women, family...) in your community know how to detect the signs of labour? Do they know what the future mother should do when labour starts?
4. Do people in your community know how to care for a woman during her labour, delivery and after the birth?
5. Do family members know how to care for the mother's baby after its birth? Can they (mothers, family, friends...) recognize the ten signs that there may be a problem with the baby?